

Perger, Infant Minor by his Litigation Guardian Perger et
al. v. Olsen et al.

[Indexed as: Perger (Litigation guardian of) v. Olsen]

73 O.R. (3d) 102
[2004] O.J. No. 4604
Court File No. 02-0213

Ontario Superior Court of Justice,
Pierce J.
October 20, 2004

Insurance -- Automobile insurance -- No-fault regime --
Threshold requirements -- Active, athletic plaintiff suffering
soft tissue injuries -- Plaintiff's attitude towards his
injuries leading to depression and alcohol abuse and affecting
his marriage and his relationship with his children and his
friends -- Plaintiff suffering non-transitory personality
changes -- Plaintiff's attitude towards his injuries making him
thin-skulled victim -- Plaintiff's physical and psychological
injuries meeting statutory threshold -- Insurance Act, R.S.O.
1990, c. I.8, s. 267.5(5).

The plaintiff, an athletic, outgoing 45-year-old physical
education teacher, suffered soft tissue injuries in a motor
vehicle accident in May 2001. According to his chiropractors,
at the time of his personal injury trial, he still suffered an
overall 20 per cent disability. After the accident, he became
depressed and withdrawn and started abusing alcohol. His
marriage and his relationship with his children suffered. A
motion was brought to determine whether the plaintiff's
injuries constituted a "permanent serious impairment of an
important physical, mental or psychological function" as
described in s. 267.5(5) of the Insurance Act.

Held, the question should be answered affirmatively.

The pain caused by the plaintiff's soft tissue injuries might not, on its own, stand out as constituting a permanent impairment. However, the plaintiff's [page103] psychological overlay, together with his ongoing physical difficulties, combined to affect him in a more profound way. His response to his physical injuries was anger, withdrawal and depression. It was his attitude towards the injuries, rather than the injuries themselves, which made him a thin-skulled victim. The plaintiff would be required to treat his depression indefinitely. The residual physical injury suffered by the plaintiff, in combination with the psychological injury, had a profound effect on the plaintiff's well-being. His impairment altered his entire attitude to life, work, family and friends. It appeared that the change in his personality was not transitory. The injuries suffered by the plaintiff, which were both physical and psychological, constituted a serious impairment of important physical and psychological functions which were permanent.

Cases referred to

Bos v. James (1995), 22 O.R. (3d) 424, 11 M.V.R. (3d) 295, [1995] O.J. No. 598 (Gen. Div.), supp. reasons (1995), 23 O.R. (3d) 575, 11 M.V.R. (3d) 295n (Gen. Div.); Chilman v. Dimitrijevic (1996), 28 O.R. (3d) 536, 135 D.L.R. (4th) 61, 19 M.V.R. (3d) 187 (C.A.); Jones v. Mazolla, [2004] O.J. No. 366, [2004] O.T.C. 103 (S.C.J.); Meyer v. Bright (1993), 15 O.R. (3d) 129, [1993] O.J. No. 2446 (C.A.)

Statutes referred to

Family Law Act, R.S.O. 1990, c. F.3

Insurance Act, R.S.O. 1990, c. I.8, s. 267.5

MOTION to determine whether the plaintiff's injuries met the

statutory threshold.

Christopher D.J. Hacio, for plaintiffs.

John H. Hornak, for defendants.

PIERCE J.: --

Background

[1] The plaintiff sought damages for his injuries sustained in a motor vehicle accident, which were assessed by a jury. However, the results are not relevant in determining whether the plaintiff's injuries constitute a "permanent serious impairment of an important physical, mental or psychological function", as described in s. 267.5(5) of the Insurance Act, R.S.O. 1990, c. I.8.

[2] The plaintiff was 45 at the time of the accident, and 48 at trial. He was "T-boned" at an intersection in Thunder Bay, Ontario, when the defendant went through a red light. Liability was admitted. The accident occurred on May 16, 2001.

[3] Mr. Perger is married, and the father of two sons, Christopher, aged 20 at the time of trial, and Andrew, 18. The family made claims under the Family Law Act, R.S.O. 1990, c. F.3, for the loss of Mr. Perger's care, guidance and companionship.

[4] Before the accident, the plaintiff, trained as a physical education teacher, was teaching co-operative education at the [page104] high school level, and coaching at least two sporting teams each year, including track and field and volleyball. He also coached other sports, and in the past, coached minor hockey, including teams on which his sons played. His approach to coaching was to demonstrate skills, particularly in throwing events for track and in hockey. As a younger man, he played university hockey.

[5] Before the accident, Mr. Perger was an energetic man, who enjoyed playing pick-up hockey, tennis and golf with his children, lifting weights and taking several long walks each week, both with his friend and his wife. The Pergers might walk for three hours a day on weekends. He was also active doing yard work, handyman tasks and motor vehicle repair. He enjoyed watching sports on TV, especially with the children. In the community, the Pergers went to movies and socialized with friends and family.

[6] Summers were spent fishing, camping and enjoying activities at Lac des Milles Lac, including campfires and visits with friends and Ms. Perger's family. Mr. Perger also enjoyed a vigorous annual golf weekend with colleagues. In alternate years, the family would travel by car to Kelowna, B.C. to visit Mr. Perger's parents and brothers who live there. His personality before the accident was described as outgoing and his attitude positive. A performance appraisal done at work in 1995 praises Mr. Perger's contribution to the school.

[7] The plaintiff testified he enjoyed his job, placing and supervising students in job experiences in the city, and conducting work readiness classes. He felt he was helping students consider their futures. He was on his way to a supervisory session when he was involved in the collision.

[8] School records show that he was off sick 17.5 days during the 1998-99 school year; 11.5 days during 1999-2000; 17 days in 2000-2001 (taking only three days off after the accident); and 26 days during 2001-2002, with a concentration of days being taken after he complained to his family doctor of difficulty sleeping in April 2002. He took all but a few weeks off during the 2002-2003 school year, returning to work full time in June of 2003.

[9] After the collision, Mr. Perger felt dazed and experienced a little pain in his neck and back. This became worse as the day went on. His neck, lower back and jaw were painful, his right wrist was strained, and he had [a] headache. Although he described feeling worse the next day, he testified he went to work. He did not seek medical attention until nine

days after the accident, when he went to [a] chiropractor, Dr. Dana Gleeson.

[10] She assessed him, and began treating his neck, shoulders and lower back with massage, ice, ultrasound and chiropractic [page105] manipulation. Mr. Perger saw Gleeson four times in the two weeks after the accident.

[11] The Pergers decided in May of 2001 to give up their campsite at Lac des Milles Lacs. The defence contends it was because of the cost, nearly \$2,000 per year, but the plaintiff testified it was because his time was occupied with medical appointments.

[12] Mr. Perger continued chiropractic treatment, which brought him relief from symptoms. There were seven appointments in June 2001, 12 appointments in July, six in August, seven in September, five in October, four in November, and three in December. By contrast, the plaintiff did not see his family doctor, Dr. deBakker, until mid-August 2001, and then his visit was unrelated to his symptoms from the accident.

[13] In 2002, Mr. Perger's appointments with Dr. Gleeson were as follows: three in January, two in February, four in March, four in April, three in May, four in June, five in July, two in August, and five in September.

[14] That September the plaintiff also began the first of 47 rehabilitative appointments with the Natural Health and Healing Centre. There he worked on increasing his range of motion and muscle strength, at the suggestion of Dr. Gleeson. He continued this rehabilitation program until he was discharged on May 6, 2003 as being, on average, 80 per cent improved. This conclusion is based largely on the plaintiff's self-reports. When Mr. Perger first attended this program, he was assessed as having a third-degree whiplash injury, whereas his own chiropractor classed his injury as second-degree whiplash. The plaintiff continued to work out at the rehabilitation facility after his discharge, but never took advantage of their offer of a free reassessment.

[15] In September of 2002, the dentist, Dr. Adams, examined Mr. Perger for pain at the left temporomandibular joint in his jaw and recommended a splint. The plaintiff has never pursued this, although it was expected the cost would be covered by his medical plan. At that time, Mr. Perger intended to discuss realignment of his jaw with an orthodontist, but he never did so. Dr. Adams could not be definitive that a splint would improve the jaw symptoms of pain, locking and clicking, but testified that it offers relief to many patients and is a less intrusive treatment than surgery.

[16] The plaintiff decided to stay home from work during the 2002-2003 academic year, returning in June of 2003. He did not lose income for this decision, as he was able to draw on his sick days through work.

[17] In October of 2002, the plaintiff spent several days at the Oakville Sleep Clinic, where a diagnosis of sleep apnea was [page106] made. This is unrelated to the accident, in the opinion of the plaintiff's psychiatrist, Dr. Allain, whose opinion I accept. She testified that sleep disorder may be a symptom of depression, but this does not encompass sleep apnea. Mr. Perger tried a machine to remedy his sleep apnea, but could not tolerate it, and abandoned the effort. In due course, when he lost weight, the problem disappeared.

[18] The plaintiff's medical records document that he complained to Dr. deBakker of sleeping difficulties as early as 1997. However, the plaintiff maintains his sleeping difficulties are related to the accident. The first medical record of any complaint of this kind occurs three months after the accident, when Dr. deBakker gave him a prescription for sleeping medication.

[19] Mr. Perger's deviated septum, diagnosed by Dr. Turner, the ear, nose and throat specialist, is also unrelated to the accident. It contributed to his breathing difficulties while sleeping.

[20] In November 2002, the plaintiff had 11 chiropractic visits, and saw his family doctor twice for depression. This is

the first time since the accident that the plaintiff was treated for depression. However, Mr. Perger was treated for a reactive depression in February 1998, when his wife was severely injured in a car accident, and the plaintiff was having difficulty coping with working, running the household and caring for his wife and children.

[21] After his accident, the plaintiff did not consult deBakker regarding his physical symptoms, advising him only that he was satisfied with chiropractic treatment. At no time was he referred to a pain management clinic or to an orthopedic specialist concerning his physical injuries. Mr. Perger reported doing well with his physical symptoms in November of 2002, but depression was then becoming a greater concern. The first anti-depressant Dr. deBakker gave him made him groggy and was changed two months later.

[22] As a result of Mr. Perger admitting to his family doctor that he was drinking to excess, he was referred to the Catholic Family Centre for counselling. He was counselled and further referred to the Family Services Centre for help with his drinking.

[23] The plaintiff denied that his drinking problems stemmed from the accident. He acknowledged that his abuse of alcohol caused considerable distress to his wife and children. However his psychiatrist expressed the opinion that his depression and heavy drinking were related, tied in to pain and stress.

[24] Chiropractic visits continued on at least a weekly basis through the first quarter of 2003, with occasional appointments with deBakker concerning depression or problems unrelated to [page107] the accident. By April, Mr. Perger reported he felt 80 per cent recovered from his physical symptoms, and wanted to return to work on a part-time basis. He felt he had made a maximum recovery. When he filled out an intake form for Family Services that month, he made no mention of accident-related injuries when asked about his health.

[25] During the next quarter, chiropractic sessions averaged one a week. His family doctor saw him monthly, on average,

concerning excessive drinking, depression, family and financial problems as well as unrelated complaints. Despite telling several health care providers he was having financial problems, the plaintiff denied financial trouble at trial and admitted he had suffered no loss of income because of the accident.

[26] April of 2003 saw the start of eight months of sobriety, with a reduction in the stresses in the family. On June 4, Mr. Perger returned to work for the balance of the school year.

[27] Through the balance of 2003, the plaintiff received chiropractic treatment up to once a week, although some months it was less. By October of 2003, Mr. Perger reported to Dr. deBakker that he was sober, back at school and doing well, though continuing to work out and take chiropractic treatment. He was still on anti-depressants.

[28] During 2004, the chiropractic treatments were generally about three per month, although in March they were as high as five and July as low as one. In September, there were none. Mr. Perger flew alone to Kelowna in the summer, and was more active, telling his psychiatrist he golfed 20 times. He also resumed tennis.

[29] Alcohol abuse is a significant problem for Mr. Perger. The plaintiff resumed drinking after Christmas in 2003, a problem that is ongoing, and generating much stress in the family. He pleaded guilty to a charge of impaired driving in the spring of 2004, was fined, and lost his licence for a year. A major stressor for the family is driving him to his school and other appointments, an issue that still creates tension in the family.

[30] In April of 2004, plaintiff's counsel referred him to Dr. Allain, for psychiatric assessment and report. Dr. Allain testified as an expert at the trial. She saw the plaintiff in August and also in September. She advised him not to drink while on medication; his pharmacist gave him the same instructions. The plaintiff understands that this is a major source of marital and family discord, but he continues to drink.

[31] The plaintiff told his psychiatrist that he had financial difficulties, a proposition she accepted, unaware that he suffered no loss of income. At trial, Dr. Allain agreed that any [page108] financial stressors were unrelated to the accident. Nor did the plaintiff tell Dr. Allain that he had been charged with impaired driving in [the] previous month when he saw her in April. It was her opinion at that time that the plaintiff's depression was resolving, and that he abused alcohol, but this was partly in remission.

[32] Dr. Allain's report of April 24, 2004 concluded:

His depression and pain will likely improve although they remain three years after the accident and they may persist for a long time. He will likely need ongoing medications and there is the possibility that his depressive symptoms could reoccur.

[33] She opined that the plaintiff's depression was partly pain related, and partly his reaction to stress. She was of the view that the accident contributed to these symptoms. She felt his prognosis was more guarded because he continued to drink. Dr. Allain agreed he would function better and sleep better if he did not drink. She opined that disrupted sleep may be a symptom of depression.

[34] It is an open question as to whether the abuse of alcohol generates stress that feeds depression or is a symptom of it. Certainly there is evidence that the consequences of excessive drinking have created more stress for the plaintiff. The loss of a driver's licence is an example. The spending of money on alcohol and entertainments while drunk is another. The defence has argued vigorously that Mr. Perger should not be compensated for his abuse of alcohol, given his position that it is not accident related.

[35] There was evidence at trial that the plaintiff was anxious about losing his vision as a result of a cataract problem. This is not accident related, and to the extent that it forms part of his depression, cannot be considered.

[36] Following the accident, Mr. Perger withdrew socially from family, colleagues and friends. He tended to isolate himself and is sad and negative. He was irritable, argumentative with his wife, and confrontational with others. There is much tension in the home. His children avoid him and try to assist their mother by doing chores their father used to do. Ms. Perger has repeatedly threatened to take the boys and leave if he continues to drink. He seldom goes fishing and has not returned to camping.

The Law

[37] The plaintiff submits that he suffers a permanent serious impairment of an important physical, mental, or psychological function based on his physical injuries from the car accident, and [page109] the depression that flowed from being injured. He bears the onus of bringing himself within one of the statutory exceptions, on a balance of probabilities. See *Chilman v. Dimitrijevic* (1996), 28 O.R. (3d) 536, 135 D.L.R. (4th) 61 (C.A.).

[38] The leading discussion of these issues is found in *Meyer v. Bright* (1993), 15 O.R. (3d) 129, [1993] O.J. No. 2446 (C.A.). However, it must be remembered that Meyer fell under the Ontario Motorists' Protection Plan, a legislated scheme that enhanced no-fault benefits to injured motorists payable by the victim's own insurer. Improved no-fault benefits were offset by the limited right of the injured party to sue a tortfeasor. The language of the statute restricted suits to death, permanent serious disfigurement, or

permanent serious impairment of an important bodily function caused by continuing injury which is physical in nature.

[39] The Bill 59 terminology, that applies to the case at bar, limits suits to death, permanent serious disfigurement, or

permanent serious impairment of an important physical, mental, or psychological function.

Thus, the cases must be read with the statutory differences in mind. Counsel agree that the seriousness of an injury relates to the degree of impairment arising from it, and not from the injury itself. An impairment is serious if it substantially interferes with the plaintiff's usual daily activities or the continuation of his employment. Necessarily, the degree of severity will be personal to each individual, and must be measured against his condition and situation in life. See Meyer, supra.

[40] Counsel concur that the term "permanent" does not mean "forever", but rather a condition that lasts for an indefinite period of time. See Jones v. Mazolla, [2004] O.J. No. 366, [2004] O.T.C. 103 (S.C.J.) and Bos v. James (1995), 22 O.R. (3d) 424, [1995] O.J. No. 598 (Gen. Div.).

Discussion

[41] The chiropractic evidence defines the plaintiff as having sustained a whiplash-associated disorder of the second or third degree. The reports of the plaintiff's physical difficulties rely almost completely on the plaintiff's self-report of his symptoms of neck, back and shoulder pain, and headache.

[42] Virtually the only physical symptom the plaintiff complained about that was observable to health care providers was the injury to his left temporomandibular joint ("TMJ") in the jaw. In that instance, clicking and stiffness in the left jaw was discernible. [page110] Mr. Perger's dentist described the condition as mild and recommended a splint be worn at night. As this was never pursued in the two years before trial, I conclude the symptoms have resolved, or at least cause minimal discomfort.

[43] I accept the evidence of Dr. Allain that the sleep apnea and sleep disturbance from the deviated septum is not accident related.

[44] The defence argued vigorously that the plaintiff recovered fully from his injuries by two years after the

accident; as he can work and perform the tasks of daily living, he does not meet the statutory exemption. It submits that the "absolute absence of diagnostic data", lack of evidence concerning chronic pain, and lack of prognosis concerning physical injury does not meet the test.

[45] The evidence of the chiropractor, Dr. Schroeder, was that soft tissue injuries can be debilitating and difficult to treat. He stated that once damaged, muscles, tendons and ligaments don't heal like bones, and when they do heal, there is often scar tissue. He added, it is difficult to assess or understand the person injured in this way.

[46] While there is force in the defence argument that the plaintiff is a poor historian and prone to exaggeration, there is more objective evidence available about the impact of this accident on his physical state. His friend, and fellow teacher, Tony Garofolo, testified in the plaintiff's absence. His evidence was given in a considered and credible way about the changes in Mr. Perger since the accident. The two men coached together, and were often in each other's company as friends and colleagues; they knew each other at home as well as in school. As a trained physical education teacher with similar interests to the plaintiff's, Mr. Garofolo was a well-positioned observer. Since the accident, the Pergers' younger son has sought refuge in the friendship of Garofolo's son, and in the Garofolo home. Since the accident, the men's friendship has suffered, and they are not as close.

[47] Mr. Garofolo still walks occasionally with the plaintiff. Unlike his pre-accident gait, Mr. Perger now is unstable walking when there are changes in sidewalk elevation. He walks around puddles. His gait is also very awkward going upstairs. Garofolo has seen the plaintiff taking pills.

[48] Before the accident, Mr. Perger's style of coaching was to demonstrate a skill for the students. For example, in the throwing events in track, Mr. Perger would demonstrate the shotput, discus, or javelin. He no longer does this, but adopts a more passive approach. He has the assistant coaches set up equipment. [page111]

[49] The Perger children also testified with their parents absent from the courtroom. Previously, when coaching hockey, Mr. Perger would have raced his sons, and enjoyed the exercise. He played scrub games with the neighbourhood boys. Andrew Perger testified that his father attempted skating once since the accident, then took his skates off and looked upset. By the summer of 2004, a game of golf consisted of nine holes, rather than the customary 18. Christopher Perger described his father being slower and less mobile playing tennis. He also observed his father walking less with his mother.

[50] These observations of others corroborate the plaintiff's self-reports of continuing injury. They support the conclusion of the chiropractors that Mr. Perger still suffers an overall 20 per cent disability. He has not made a complete recovery.

[51] The defence cites this para. at p. 150 O.R. of Meyer, *supra*:

Because it is only a serious impairment which will qualify as an exception under s. 266(1)(b), it is apparent that the Legislature intended that injured persons are required to bear some interference with their enjoyment of life without being able to sue for it

[52] The defence, quite properly, seeks to evaluate the weaknesses in the plaintiff's case by looking at each symptom in isolation. However, in considering the impact of an accident on a person's health and functioning, the court must look at the cumulative effect on a person's life. A sore neck or a sore back, viewed in isolation, may not meet the statutory exemption, but when combined with other difficulties, the court may reach a different conclusion.

[53] That is the case here. The pain caused by the plaintiff's soft tissue injuries may not, on its own, stand out as constituting a permanent impairment. The plaintiff has returned to work full time. He does enjoy some recreation, albeit on a greatly reduced scale, and with some physical cost. But the witnesses who knew him before and after the accident

make it clear he is not the same man. It may be that the psychological overlay, together with his ongoing physical difficulties, have combined to affect this plaintiff in a more profound way.

[54] I am satisfied that there is impairment related to the plaintiff's physical, mental and psychological functions. It is evident that the residual injuries he still suffers, which some might consider minor, go to the heart of the plaintiff's identity. Before the accident, Mr. Perger took pride in his physical fitness. He not only enjoyed athletic pursuits with his students, family and friends, these activities defined him as a man. He wanted to be a model physical education teacher. He wanted his family to rely on him, to the point that the family no longer went camping [page112] because he couldn't do the heavy work involved in packing the family to go. The Perger children, now 18 and 20, could certainly have loaded the gear required but that adaptation would have been an assault on Mr. Perger's self-image.

[55] Each individual's reaction to injury is different. Some might look for other, less taxing forms of recreation and make a good adjustment to his or her limitations. Mr. Perger, a private man, has not been able to do that. He has not been able to tell his children that his injuries are the reason he has withdrawn from involvement with them. He did not tell his best friend about the accident until several days later. He has hidden his concerns away. His response has been anger, withdrawal and depression. It is his attitude to the injury, rather than the injury itself, which makes him the [thin-skulled] victim.

[56] Despite Mr. Perger's evidence to the contrary, I accept that the plaintiff's abuse of alcohol is accident related. So is his depression. Tony Garofolo testified that abuse of alcohol is something new in the plaintiff's behaviour since the accident. Garofolo described the pre-accident Perger as being spontaneous, generally optimistic, and at the centre of workplace gatherings, telling stories and joking. His drinking did not stand out at social gatherings. He had a strong relationship with his wife and children and spent time with

them.

[57] After the accident, he described the plaintiff's negativity, and withdrawal from social activity, including with the Perger children. He stated frankly that the plaintiff's negative attitude is tiring, such that he needs to take a break from their relationship now. He testified that if Mr. Perger now has more than two drinks, he seems "to lose it". He recalled driving him home from a meeting at which the plaintiff had two drinks; the plaintiff asked him to stop at a bar on the way home but Garofolo declined. It was evident that Mr. Garofolo misses the Robbie Perger who had formerly been his friend. It was evident that he was shocked at the distress Ms. Perger expressed to his wife about the marriage, knowing what a private person she is.

[58] The Perger boys described the ongoing tension in their home, to the extent that instead of seeking their father out as a role model, they now avoid him. Andrew Perger tearfully described having lost his best friend. Ms. Perger stated her husband was not the man she married. The family now tries not to aggravate him when he is having bad days.

[59] According to Dr. Allain, Mr. Perger is aware, on an intellectual level, of the stress he generates in the family by abusing alcohol, but he continues to drink to excess. I accept her opinion that the depression and the drinking feed on each other. I [page113] accept her opinion that his prognosis is more guarded given that he continues to drink. I accept that the plaintiff will be required to treat his depression with medication indefinitely. If he does improve, Dr. Allain testified that there is a risk the depression will recur.

[60] It is the residual physical injury suffered by the plaintiff in combination with the psychological injury that has had such a profound effect on the plaintiff's well-being. An impairment is serious if it substantially interferes with the plaintiff's usual daily activities. Such is the case here. Mr. Perger's impairment alters his entire attitude to life, work, his family and friends. For this reason, it is serious. Madam Justice Herman observed at para. 44 in *Jones v. Mazolla*, supra:

The ability to sleep is important. So, too, is the ability to care for one's children, share activities with them, and otherwise lead a normal life without significant discomfort.

[61] Is the function that is affected important? Yes. In this case, the primary impairment is to the plaintiff's mental health. It has therefore altered his entire familial relationship. It has altered his self-esteem. It changed his approach to his work. He is left with residual physical problems that interfere significantly with sporting endeavours, by which the plaintiff realized much joy in life. It has caused him to withdraw from social interaction and compensate by abusing alcohol. That alone may cost him his marriage.

[62] Does the impairment have the degree of permanence that brings it within the statutory exception? In my view, it does. Trial took place approximately 3 1/2 years after the accident. Third-party evidence corroborates the continuation of physical injury. The chiropractors do not anticipate further improvement. The depression symptoms show up in the family doctor's chart about two years before trial. Treatment has been ongoing since then as the symptoms continue. Dr. Allain testified that the plaintiff will require medication for depression indefinitely. An indefinite course of mood-altering medication is a significant event in any life. It appears that the change in Mr. Perger's personality is not transitory.

[63] On a balance of probabilities, I therefore conclude that the injuries suffered by Robbie Perger, which are both physical and psychological, constitute a serious impairment of important physical and psychological functions, which are permanent. A declaration will issue accordingly.

Order accordingly.

[page114]